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April 20, 2026

COMMITTEE SUBSTITUTE
FOR ENGROSSED
HOUSE BILL NO. 3650

By: Stinson of the House

and

Rosino of the Senate

An Act relating to the state Medicaid program; amending 56 O.S. 2021, Section 4002.12, as last amended by Section 7, Chapter 448, O.S.L. 2024 (56 O.S. Supp. 2025, Section 4002.12), which relates to minimum rates of reimbursement; extending certain termination dates; establishing certain reimbursement rates for multistate contracts; authorizing promulgation of certain rules; defining terms; and providing an effective date.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 56 O.S. 2021, Section 4002.12, as last amended by Section 7, Chapter 448, O.S.L. 2024 (56 O.S. Supp. 2025, Section 4002.12), is amended to read as follows:

Section 4002.12. A. Until ~~July 1, 2027~~ July 1, 2028, the Oklahoma Health Care Authority shall establish minimum rates of reimbursement from contracted entities to providers who elect not to enter into value-based payment arrangements under subsection B of this section or other alternative payment agreements for health care

1 items and services furnished by such providers to enrollees of the
2 state Medicaid program. Except as provided by subsection I of this
3 section, until ~~July 1, 2027~~ July 1, 2028, such reimbursement rates
4 shall be equal to or greater than:

5 1. For an item or service provided by a participating provider
6 who is in the network of the contracted entity, one hundred percent
7 (100%) of the reimbursement rate for the applicable service in the
8 applicable fee schedule of the Authority; or

9 2. For an item or service provided by a non-participating
10 provider or a provider who is not in the network of the contracted
11 entity, ninety percent (90%) of the reimbursement rate for the
12 applicable service in the applicable fee schedule of the Authority
13 as of January 1, 2021.

14 B. A contracted entity shall offer value-based payment
15 arrangements to all providers in its network capable of entering
16 into value-based payment arrangements. Such arrangements shall be
17 optional for the provider but shall be tied to reimbursement
18 incentives when quality metrics are met. The quality measures used
19 by a contracted entity to determine reimbursement amounts to
20 providers in value-based payment arrangements shall align with the
21 quality measures of the Authority for contracted entities.

22 C. Notwithstanding any other provision of this section, the
23 Authority shall comply with payment methodologies required by
24 federal law or regulation for specific types of providers including,

1 but not limited to, Federally Qualified Health Centers, rural health
2 clinics, pharmacies, Indian Health Care Providers and emergency
3 services.

4 D. A contracted entity shall offer all rural health clinics
5 (RHCs) contracts that reimburse RHCs using the methodology in place
6 for each specific RHC prior to January 1, 2023, including any and
7 all annual rate updates. The contracted entity shall comply with
8 all federal program rules and requirements, and the transformed
9 Medicaid delivery system shall not interfere with the program as
10 designed.

11 E. The Oklahoma Health Care Authority shall establish minimum
12 rates of reimbursement from contracted entities to Certified
13 Community Behavioral Health Clinic (CCBHC) providers who elect
14 alternative payment arrangements equal to the prospective payment
15 system rate under the Medicaid State Plan.

16 F. The Authority shall establish an incentive payment under the
17 Supplemental Hospital Offset Payment Program that is determined by
18 value-based outcomes for providers other than hospitals.

19 G. Psychologist reimbursement shall reflect outcomes.
20 Reimbursement shall not be limited to therapy and shall include but
21 not be limited to testing and assessment.

22 H. Coverage for Medicaid ground transportation services by
23 licensed Oklahoma emergency medical services shall be reimbursed at
24 no less than the published Medicaid rates as set by the Authority.

1 All currently published Medicaid Healthcare Common Procedure Coding
2 System (HCPCS) codes paid by the Authority shall continue to be paid
3 by the contracted entity. The contracted entity shall comply with
4 all reimbursement policies established by the Authority for the
5 ambulance providers. Contracted entities shall accept the modifiers
6 established by the Centers for Medicare and Medicaid Services
7 currently in use by Medicare at the time of the transport of a
8 member that is dually eligible for Medicare and Medicaid.

9 I. 1. The rate paid to participating pharmacy providers is
10 independent of subsection A of this section and shall be the same as
11 the fee-for-service rate employed by the Authority for the Medicaid
12 program as stated in the payment methodology in OAC 317:30-5-78,
13 unless the participating pharmacy provider elects to enter into
14 other alternative payment agreements.

15 2. A pharmacy or pharmacist shall receive direct payment or
16 reimbursement from the Authority or contracted entity when providing
17 a health care service to the Medicaid member at a rate no less than
18 that of other health care providers for providing the same service.

19 J. Notwithstanding any other provision of this section,
20 anesthesia shall continue to be reimbursed equal to or greater than
21 the anesthesia fee schedule established by the Authority as of
22 January 1, 2021. Anesthesia providers may also enter into value-
23 based payment arrangements under this section or alternative payment
24 arrangements for services furnished to Medicaid members.

1 K. The Authority shall specify in the requests for proposals a
2 reasonable time frame in which a contracted entity shall have
3 entered into a certain percentage, as determined by the Authority,
4 of value-based contracts with providers.

5 L. Capitation rates established by the Oklahoma Health Care
6 Authority and paid to contracted entities under capitated contracts
7 shall be updated annually and in accordance with 42 C.F.R., Section
8 438.3. Capitation rates shall be approved as actuarially sound as
9 determined by the Centers for Medicare and Medicaid Services in
10 accordance with 42 C.F.R., Section 438.4 and the following:

11 1. Actuarial calculations must include utilization and
12 expenditure assumptions consistent with industry and local
13 standards; and

14 2. Capitation rates shall be risk-adjusted and shall include a
15 portion that is at risk for achievement of quality and outcomes
16 measures.

17 M. The Authority may establish a symmetric risk corridor for
18 contracted entities.

19 N. The Authority shall establish a process for annual recovery
20 of funds from, or assessment of penalties on, contracted entities
21 that do not meet the medical loss ratio standards stipulated in
22 Section 4002.5 of this title.

23 O. 1. The Authority shall, through the financial reporting
24 required under subsection G of Section 4002.12b of this title,

1 determine the percentage of health care expenses by each contracted
2 entity on primary care services.

3 2. Not later than the end of the fourth year of the initial
4 contracting period, each contracted entity shall be currently
5 spending not less than eleven percent (11%) of its total health care
6 expenses on primary care services.

7 3. The Authority shall monitor the primary care spending of
8 each contracted entity and require each contracted entity to
9 maintain the level of spending on primary care services stipulated
10 in paragraph 2 of this subsection.

11 P. 1. Notwithstanding the provisions of subsection A of this
12 section but subject to all other provisions of this section, for an
13 item or service delivered under a multistate contract, the
14 reimbursement rate for that item or service shall be the lesser of:

15 a. the rate specified in the multistate contract, or

16 b. the applicable rate in the fee schedule of the
17 Authority.

18 2. The Oklahoma Health Care Authority Board may promulgate
19 rules to implement the provisions of this section including, but not
20 limited to, rules that further define the terms used in this
21 subsection or that set limitations or restrictions around the
22 applicability of the provisions of this subsection.

23 3. As used in this subsection:
24

- 1 a. "multistate contract" means a contract that a
2 contracted entity or its parent company holds with
3 another entity under which the entity provides health
4 care services on behalf of the contracted entity
5 throughout a service area that includes this state and
6 at least one other state, and
7 b. "parent company" means any company that directly or
8 indirectly controls a contracted entity.

9 SECTION 2. This act shall become effective November 1, 2026.

10 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
11 April 20, 2026 - DO PASS AS AMENDED BY CS
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